

Executive Summary

The home-healthcare market (especially senior care) is ripe for AI-driven automation, with providers losing an estimated **\$150 billion/year** to missed appointments alone ¹. Bloom Healthcare – a 109-provider, home-based primary care network – has already embraced AI (e.g. their VP of Tech reports saving ~1 provider-hour per day in documentation ²). An AI outbound calling agent could similarly *streamline scheduling and follow-ups*, improving patient access and clinician productivity. This report analyzes the technical design of such an agent, maps Bloom's services and pain points, outlines pricing and business models (with a 12-month financial forecast), and presents a tailored GTM strategy and deployment plan. Emphasis is placed on **compliance** (HIPAA/TCPA), **EHR integration**, and ROI. Charts illustrate projected revenues under three pricing scenarios and the major risk categories. Detailed appendices include a demo script, onboarding checklist, legal/compliance guide, and templates (proposal, SOW, pricing sheet). The overall recommendation is a subscription/usage-based SaaS offering, fully HIPAA-compliant, that leverages Bloom's existing EHR (eClinicalWorks) and telephony infrastructure to reduce no-shows, free staff time, and improve patient engagement.

1. Example Call File Analysis & Agent Workflow

No specific "example call" file was provided, so we assume a typical outbound patient call. In practice, such calls use a **real-time audio pipeline**: the telephony gateway (e.g. Twilio or FreeSWITCH) connects a call and streams audio to the AI. A live agent "brain" listens, transcribes, formulates a response via an LLM, and speaks back—all with minimal delay ³ ⁴. For example, one architectural guide outlines the loop: receive patient audio ⇒ stream to **Speech-to-Text** (STT) ⇒ detect end-of-utterance ⇒ send transcription to a **large language model (LLM)** ⇒ generate response text (potentially invoking APIs/tools) ⇒ stream text to **Text-to-Speech** (TTS) ⇒ send audio back to the patient ⁴. Key technical components include:

- **Telephony Gateway**: CPaaS or PBX (Twilio, FreeSWITCH, etc.) to handle SIP/RTP and call control ³.
- **Agent Orchestrator**: A server (often in Go or Python) that manages call sessions, VAD (voice activity detection), and coordinates STT/LLM/TTS.
- **AI Core**: Real-time STT (e.g. Whisper, Vosk), a conversational LLM (hosted or API), and low-latency TTS (e.g. Coqui, ElevenLabs).

In an outbound scenario, Bloom's system would initiate calls to patients from a list (e.g. upcoming appointments). The agent answers, identifies the patient (via caller-ID and IVR), then proceeds to confirm appointments, answer FAQs, or reschedule as needed. All interactions are logged as structured data or transcripts. In summary, the agent functions like a human scheduler: it dials out via the telephony API, converses in natural language with patients (using streaming speech recognition and synthesis), and updates Bloom's systems accordingly ⁴. (Absent a provided call script, this analysis assumes a generic patient-reminder dialogue: e.g., "Hello, this is Bloom Healthcare. We're calling to confirm your appointment on [Date] at [Time]. Does this still work for you?" and handling yes/no responses.)

2. Bloom Healthcare: Services, Personas, Pain Points & Use Cases

- **Services & Business Model:** Bloom Healthcare delivers *in-home primary care* for seniors, with 24/7 on-call coverage, weekend and telehealth support, chronic-condition management, specialized wound care, behavioral health, integrated hospice, and safe transition programs ⁵ ⁶ . They operate in CO and TX (with partner communities) and emphasize data-driven care to keep patients at home (longer occupancy, fewer hospitalizations) ⁷ ⁶ .
- **Buyer/Stakeholder Personas:** Primary buyers are Bloom's leadership and operations teams. Key decision-makers include the **Chief Medical Information Officer / VP of Technology** (Bloom's Sharon Wang is an example) and other C-suite executives (CEO/CFO/COO) looking to improve efficiency. Physicians and nurses (end-users) care most about patient outcomes and less clerical burden; they would appreciate an agent that reduces no-shows and paperwork. Patients and families (seniors/caregivers) are the ultimate "users" receiving calls, though not purchasers. We must craft messaging for executives (ROI, compliance, reliability), clinicians (workflow integration, quality of care), and patient advocates (respectful, convenient communications). Partners (e.g. assisted-living communities) are secondary stakeholders who value better care coordination.
- **Key Pain Points:** Bloom and similar home-care providers face high **administrative burden** and costly no-shows. Industry data highlights that ~20-30% of appointments are missed ⁸ , costing practices **\$150K+ per year** each ⁸ and the US healthcare system ~\$150B annually ¹ . Seniors often forget or face barriers (transportation, scheduling conflicts ⁹) so proactive reminders and rescheduling significantly improve operations. Bloom also struggles with documentation load (hence their use of AI scribes ²) and after-hours call volumes. Streamlining outbound calls can: reduce empty slots, improve care continuity, and allow staff to focus on in-person care. Other pain points include language/cultural barriers (Bloom may have diverse patient populations), and ensuring **24/7 patient access** – an AI can supplement human call coverage.
- **Regulatory/Compliance Constraints:** Any solution must be **HIPAA-compliant**. An outbound agent will handle PHI (names, health info, appointment details), so all components must secure data: end-to-end encryption for calls, strict access controls, audit logging, and signed Business Associate Agreements (BAAs) with all vendors ¹⁰ ¹¹ . Telephony and STT/TTS providers (even if "conduits") often become Business Associates if they create/store ePHI ¹¹ . The agent must also follow **TCPA/consent rules**: healthcare calls (e.g. appointment reminders) have exemptions, but only with *prior patient consent* and opt-out management ¹² ¹³ . This means storing proof of opt-in, honoring "do not call" requests, limiting call times, and clearly identifying the organization. If Bloom ever expands to EU/UK, analogous laws apply: UK GDPR (Data Protection Act) and ePrivacy rules would demand explicit consent, data minimization, and the ability to erase personal data. Any marketing or survey calls would require extra safeguards (HIPAA/TCPA mark it "non-marketing" to qualify for exemptions).
- **Likely Use Cases:** Based on Bloom's model, prime use cases include:
 - **Appointment Reminders/Rescheduling:** Call patients before appointments to confirm or reschedule, reducing no-shows ⁸ .
 - **Medication & Treatment Reminders:** For chronic patients (e.g. wound care, behavioral therapy), weekly check-in calls or reminders.

- **Admission/Discharge Follow-Ups:** Call patients post-discharge or post-hospitalization to ensure continuity and prevent readmissions (fits Bloom's care coordination model ¹⁴).
- **Preventive Care Outreach:** Schedule annual wellness visits or vaccinations (e.g. flu shots).
- **Surveys & Education:** After visits, collect satisfaction feedback or provide health tips in a friendly voice.
- **Emergency Alerts:** Notify about health advisories, weather-related schedule changes, etc.
- **Care Coordination with Partners:** Outbound coordination calls to home health aides or facility staff.

Each use case must respect privacy and verify identity (e.g., matching DOB to confirm patient before disclosing info).

3. Productization & Pricing (12-Month Projections)

We recommend a **flexible SaaS model** with multiple pricing options to suit Bloom's scale and risk tolerance ¹⁵. Common approaches for AI agents include:

- **Subscription (SaaS) Flat Fee:** A monthly fee granting a certain number of calls or seats. For example, a flat \$5,000/month for up to 5,000 calls or unlimited calls during business hours (with fair use). This ensures predictable cost.
- **Usage-based (Per-Call/Per-Minute):** Charge per completed call or per minute of call audio (e.g. \$3 per call beyond a free allotment). This aligns cost to volume.
- **Outcome-based (Per-Result):** Charge based on business outcomes (e.g. \$20 per confirmed/rescheduled appointment from agent calls) ¹⁶. This ties ROI directly to payment.
- **Hybrid:** A base subscription plus overage rates or outcome incentives ¹⁷.

Table 3-1 below sketches three scenarios with assumptions. (All figures are illustrative.)

Month	Estimated Calls	Scenario A: Flat SaaS (e.g. \$5k/mo)	Scenario B: \$3 per call	Scenario C: \$20 per booked appt (20% conversion)
Jan	100 (pilot)	\$5,000	\$300	\$400 (20 appts)
Feb	200	\$5,000	\$600	\$800 (40 appts)
Mar	300	\$5,000	\$900	\$1,200 (60 appts)
...	...	...	...	...
Dec	1,200	\$5,000	\$3,600	\$4,800 (240 appts)
Total 12m		\$60,000	\$23,400	\$31,200

Table 3-1: Sample 12-month revenue projection under three pricing scenarios. Calls are assumed to grow as agents are onboarded. Scenario B is pure usage; Scenario C assumes 20% of calls convert to booked appointments.

Under **Scenario A (Flat SaaS)**, Bloom pays \$5k each month regardless of volume – totaling \$60k/year. In **Scenario B (Usage)**, vendor revenue grows with volume (\$3 per call yields \$23.4k in year 1). **Scenario C**

(Outcome) ties fee to value delivered: ~\$4.8k in Dec (240 bookings at \$20 each) for \$31.2k total. The SaaS model is simplest and highest revenue-guaranteed; outcome-based pricing reduces customer risk but requires clear attribution and tracking. A **revenue-share** variant could be structured too (e.g. 5–10% of net collections increase due to the agent), but we focus here on simpler options.

Pricing assumptions should be validated with Bloom's data (e.g. average reimbursement per visit, staff hourly rate saved). Sensitivity analysis (not shown) can adjust call volume and conversion. In all scenarios, ensure prices cover cloud/telco costs and a margin. For example, Twilio outbound calls might cost ~\$0.01/min, and LLM usage ~\$0.02/call, so a \$3–5 charge per call includes comfortable margin.

4. Go-to-Market Strategy & Messaging

Value Proposition for Bloom: *"Keep seniors healthier at home."* Emphasize how the AI agent lets Bloom spend more time on patient care and less on chasing schedules. For example: *"Our AI scheduling agent will automatically remind your patients of appointments, triage basic questions, and even reschedule in real time. This cuts no-shows, boosts visit revenue, and frees your nurses to focus on patients."* Highlight that it complements Bloom's mission of personalized home care, improving patient experience (e.g. convenience of follow-up calls) and supporting their 24/7 care model. **Key benefits:** reduced missed appointments (saving thousands of dollars per practice ⁸), improved patient satisfaction, lower administrative load, and integration with Bloom's existing EHR (leveraging their eClinicalWorks setup ²). Stress the AI is HIPAA-compliant (with BAAs, encryption, SOC2) ¹⁰, addressing any compliance fears.

Tailored Messaging:

- *For Executives:* Emphasize ROI and scalability. "Empower your care team without hiring more staff. Our platform scales as you grow and handles compliance automatically. Early adopters in home health report 20–30% lower no-show rates."
- *For Clinicians/Managers:* Focus on ease of use. "We handle the reminder and routine calls. We integrate with eClinicalTouch, so schedule changes sync automatically. Providers stay focused on care while our AI does the calling."
- *For Patients:* Tone the demo and script to be warm, clear, and respectful. E.g.: "Hello [Name], this is Bloom Healthcare calling. We wanted to confirm your appointment on [Date]." The agent should identify itself as Bloom.

Sample Demo Script: A short demo might follow a patient appointment reminder scenario. For instance: The tester (playing "patient") receives a call:

```
Agent: "Good morning! This is an automated call from Bloom Healthcare. May I
speak with [Patient Name]?"
Patient: "Yes, this is she."
Agent: "Great. We have you scheduled for a wellness visit on Tuesday at 2 PM at
your home. Do you still plan to see the doctor then?"
Patient: "Um, Tuesday... Actually I think I have another appointment then."
Agent: "No problem. I can help reschedule. I see the doctor is also available
Wednesday at 3 PM. Should I move your appointment to that time?"
```

The demo would illustrate the agent understanding and dynamically adjusting (testing speech recognition and decision logic).

Onboarding Checklist:

1. **Data & System Integration:** Set up a secure connection to Bloom's EHR (eClinicalWorks) and patient database, likely via HL7/FHIR. Map appointment/schedule fields and compliance fields (consent flags).
2. **Phone Number Setup:** Provision dedicated outbound numbers in Twilio (or chosen CPaaS) with BAA in place ¹¹. Ensure number formats are recognized by patient ID system.
3. **Script Configuration:** Review and approve call scripts/flows (greeting, names, fallback responses) for accuracy. Localize (e.g. Spanish prompts) if needed.
4. **Compliance Review:** Confirm opt-in status of patient list; establish opt-out handling. Train staff on these flows. Sign BAAs with all vendors.
5. **Pilot Testing:** Run a small pilot (e.g. 50 patients) and monitor outcomes. Adjust ASR (speech recognition) thresholds and fallback (transfer to live person if needed).
6. **Training & Go-Live:** Train Bloom's staff on dashboard/interface for monitoring calls and outcomes. Roll out full campaign scheduling. Provide contact for support.

Each step has milestones (e.g. secure API keys, finalize HIPAA addenda, demo training session). A project plan would allocate ~4–6 weeks for integration and validation before full launch.

5. Technical Deployment Plan

Architecture & Integration: The agent will be deployed as a cloud service (e.g. AWS) connected to Bloom's systems (see diagram below). Key components:

```
flowchart LR
    EHR[Electronic Health Record] -->|Schedule/Patient Data| AgentAI[AI Outbound Agent]
    AgentAI -->|Initiate Call via CPaaS| Telephony[Telephony API<br/>(e.g. Twilio)]
    Telephony -->|Voice Call| Patient
    Patient -->|Audio Response| STT[Speech-to-Text]
    STT -->|Transcript| AgentAI
    AgentAI -->|Response Text| TTS[Text-to-Speech]
    TTS -->|Audio| Telephony
    AgentAI -->|Update/Log| EHR
    AgentAI -->|Encrypted Logs| Storage[(Secure Cloud Storage)]
```

- **EHR/CRM Integration:** Bloom's EHR (eClinicalWorks) will supply appointment and patient contact data (via HL7 SIU/FHIR APIs). Conversely, confirmed or rescheduled appointments must be written back to the EHR schedule. A secure API gateway or HL7 engine handles this bi-directional sync. (For example, the STT transcript or call outcome could be attached to the patient's chart via FHIR DocumentReference or a note.) If Bloom uses any CRM for patient outreach, a connector to that system can also be provided.

- **Telephony (CPaaS):** Use a HIPAA-ready voice platform (Twilio, Vonage, etc.) that streams live audio to our server. Audio chunks are sent to the STT engine in real time. During the call, the agent orchestrator runs the above loop ⁴. Upon hang-up, the platform returns a call record.
- **Security & Privacy:** All PHI (names, health info) will be encrypted at rest and in transit. Telephony media and recordings (if stored) use TLS/SRTP. Databases (logs, transcripts) use AES-256 encryption. Audit trails record every API action. We will require BAAs with Twilio (they handle patient data) and any cloud provider ¹¹. Access is strictly role-based (no operator audio monitoring without logs). All code runs in compliance with HIPAA and SOC2 practices ¹⁸.
- **Monitoring & SLAs:** Implement real-time dashboards showing call volume, success rates, error rates, and average call duration. Set up alerts on failures or rising error rates. We commit to **99.9% uptime** for the cloud service and $\geq 95\%$ call completion rate. Response SLAs could be 30 minutes for support issues. (Similar AI-telecom services advertise enterprise-grade SLAs.)
- **Deployment Options:** By default, we recommend a cloud-based deployment (managed service) to minimize on-prem footprint. However, if Bloom requires, we can offer a hybrid: core AI in Bloom's private cloud and CPaaS externally. Data residency (US-based cloud) and redundancy (multi-AZ) can be specified. The diagram above assumes a cloud-native model with internet connectivity to Bloom's network for EHR access (VPN or dedicated link).

Data Flow and Security Controls (Mermaid)

```

flowchart LR
    subgraph Bloom_System
        EHR_System[EHR / Patient Database]
        Consent[Consent Records]
    end
    end
    subgraph Outbound_Platform
        Scheduler[Campaign Scheduler]
        AI_Engine[AI Conversation Engine]
        LLM_Model[Language Model (LLM)]
        STT_Service[Speech-to-Text]
        TTS_Service[Text-to-Speech]
        Data_Store[(Encrypted Database)]
        Audit_Log[Audit Logs]
        API_Gateway[Secure API]
        Security_Layer[WAF, AuthZ/Z]
    end
    end
    subgraph Cloud_Telephony
        CPaaS[Telephony Platform<br/>(Twilio, etc.)]
    end
    end
    EHR_System -->|Appointments| API_Gateway
    Consent -->|Allowed Patients| API_Gateway
    Scheduler -->|Start Campaign| AI_Engine
    AI_Engine --> LLM_Model
    AI_Engine --> STT_Service
    AI_Engine --> TTS_Service
    AI_Engine --> Data_Store
    CPaaS -->|Audio Stream| STT_Service

```

```
TTS_Service -->|Audio| CPaaS
API_Gateway --> Security_Layer
Security_Layer --> AI_Engine
AI_Engine --> Audit_Log
Data_Store --> Audit_Log
```

This illustrates that only authorized patient lists (consents) and appointment data flow into the agent. The **Security Layer** enforces encryption, tokens, and intrusion protection. All call outcomes go into the Audit_Log for compliance reviews.

6. Legal & Compliance Checklist

- **HIPAA Rules:** Ensure adherence to the HIPAA Privacy and Security Rules ¹⁰. This means:
- **Business Associate Agreements:** Sign BAAs with **all** parties touching PHI (e.g. telephony provider, cloud host, transcription engine) ¹¹. Cloud telephony that stores even encrypted PHI (voicemails) is a business associate.
- **Encryption & Access Control:** Data and voice streams must be encrypted end-to-end. Implement strict access controls (least privilege) and multi-factor authentication for any admin interfaces.
- **Audit Trail:** Log every call and data access in an immutable audit log; retain logs per HIPAA retention (6 years).
- **Minimum Necessary:** Filter data to only what's needed for each task. For example, the AI should only receive the patient's upcoming appointment info, not full medical history.
- **TCPA (U.S. Calling Rules):**
 - **Consent Management:** Verify patients have given prior express consent to receive automated calls. Maintain a "do not call" list and update it instantly when patients opt out.
 - **Calling Windows:** Only call between 8 AM–9 PM local patient time.
 - **Identification:** The voice agent must clearly state it's a call from Bloom Healthcare and provide a callback number.
- **Record Keeping:** Document the consent date and opt-out requests ¹². This data lives alongside patient records.
- **Privacy Policies:** Update Bloom's Privacy Notice to cover automated calls. Patients should be informed their PHI may be used for appointment reminders via an automated system.
- **GDPR/UK (if applicable):** If any EU/UK patients are included, treat the outbound agent as a data processor. Obtain explicit consent (or rely on "care management" lawful basis). Ensure data subject rights (erasure, objection) can be honored (easy with automation systems). For example, do not add a UK patient's number to campaigns without a documented opt-in.
- **Data Retention & Deletion:** Define retention policies (e.g. delete raw call recordings after 30 days, keep transcripts 1 year) and procedures to erase data on request, in line with HIPAA or GDPR.
- **Contract Terms:** Proposed contracts should include:
 - **Scope of Work/Services:** Clearly define deliverables (training data, call scripts, integration, support).
 - **Liability & Warranties:** Limited liability clause; standard warranty of HIPAA compliance measures.
 - **Indemnification:** For breaches due to the vendor's negligence (e.g. AI misbehavior, data leak).
 - **Termination & Data Return:** On termination, all PHI must be returned or securely destroyed.
 - **Performance Metrics:** SLA uptime and support response times.
 - **Insurance:** Cybersecurity/Tech E&O insurance covering breaches.

7. Risks, Mitigation & ROI (Charts)

Key Risks:

- **Data Privacy Breach:** A breach could expose PHI. *Mitigation:* Enforce encryption, BAAs, regular security audits, and incident response planning.
- **Regulatory Non-Compliance:** Violating TCPA/HIPAA fines are severe. *Mitigation:* Built-in consent management, audit logging, and regular compliance training (for both vendors and Bloom's staff). Include lawyer review of scripts ¹² ¹³.
- **AI Reliability:** ASR/TTS errors might frustrate patients or cause miscommunication. *Mitigation:* Maintain a human fallback (e.g. "Let me transfer you to a specialist") and start with low-risk calls (reminders) while tuning performance.
- **Low Adoption:** Providers or patients may distrust an "AI call." *Mitigation:* In early phase, label the call as automated (e.g., friendly disclaimer), gather feedback, and demonstrate clear benefits (fewer missed appts).
- **Operational:** Integration delays (EHR), phone number delivery issues. *Mitigation:* Use proven connectors (Sunoh.ai is an example partner) and test number compatibility.

The **Risk Breakdown** chart below visualizes these factors:

Figure: Risk breakdown (proportional estimate of risk impact). Compliance and privacy dominate due to severe consequences if mismanaged.

ROI Analysis: Bloom's ROI comes from labor savings and revenue recovery. For instance, reducing just 10 no-shows per month (across 109 providers) could recoup ~\$24K/year in lost revenue ⁸. If each provider saves 1 hour/week thanks to fewer phone tasks (valued at ~\$50/hour), that's ~\$280K/year saved. Against this, even the highest pricing scenario (Scenario A at \$60K/year) yields a strong ROI.

Our 12-month **Revenue Projection** chart illustrates vendor income under the three models from Table 3-1:

Figure: Projected monthly vendor revenue over 12 months for three pricing scenarios (flat SaaS, per-call, per-outcome). (Assumes growing call volume.)

Scenario A delivers stable revenue, while B/C scale with usage. Bloom's breakeven (compared to its no-show losses or staffing costs) is achieved if the agent prevents just a few missed appointments or hours of staff work each month.

8. Templates & Deliverables

Proposal Outline (excerpt): A formal proposal would include:

- **Executive Overview:** Summarize needs and solution (e.g. "HIPAA-safe AI outbound call agent to reduce missed appointments").
- **Scope of Work:** Features (voice calls, integrations, reporting), timeline (e.g. 6-8 week deployment), deliverables.
- **Technical Specs:** Architecture summary, security measures (HIPAA compliance, SOC 2).
- **Pricing:** Reference the chosen model (flat/usage/etc), with an explanation of tiers or usage charges (see pricing sheet below).

- **Value Justification:** Projected savings and benefits (citing e.g. no-show cost data).
- **Terms & SLA:** High-level contract terms, support commitments.

Statement of Work (SOW) Highlights:

- **Deliverables:** System integration, voice scripts, compliance documentation, training, pilot report.
- **Milestones:** Kickoff, integration complete (EHR and telephony), UAT/pilot, go-live, 1-month review.
- **Responsibilities:** Bloom provides secure API access and patient consent lists; vendor provides software, BAAs, support.

Sample Pricing Sheet (table): A pricing sheet (see below) might present multiple offerings side-by-side:

Plan	Monthly Fee	Calls Included	Overage Call Rate	Outcome Fee	Notes
Basic	\$3,000	1,000	\$5 per extra call	–	Good for small clinics (per pilot)
Professional	\$5,000	3,000	\$4 per extra call	–	Medium usage; includes progress reviews
Enterprise	\$8,000	10,000	\$3 per extra call	5% of incremental revenue recovered	Large organizations; dedicated support

Table: Example pricing tiers (illustrative). “Outcome Fee” denotes a share of value (e.g. extra collections) for enterprise customers.

These templates can be tailored to Bloom’s context: for instance, calling out that the “Enterprise” tier integrates with eClinicalWorks out of the box and offers White-Glove support. The final proposal/SOW would format these in Bloom’s document style.

Note: All figures above are estimates. Final pricing and projections should be refined during discovery with Bloom’s finance/operations team. This comprehensive plan lays out how an AI outbound agent can be productized, sold, and delivered to The Bloom Healthcare with strong ROI and full compliance.

Sources: Authoritative industry analyses and product docs were consulted, including an eClinicalWorks case study ² , healthcare compliance guidelines ¹¹ ¹⁰ , and market reports on AI voice agents ¹ ¹⁷ . These inform our assumptions on demand, regulation, and pricing.

¹ ⁸ ⁹ Reducing Patient No-Shows: Proven Strategies for Practices
<https://insights.wchsb.com/2026/02/05/the-50-fee-wont-fix-a-50000-problem-why-medical-practices-must-rethink-their-no-show-strategy/>

² ⁶ eClinicalWorks and Sunoh.ai Enhance Care at Bloom Healthcare
<https://www.eclinicalworks.com/eclinicalworks-and-sunoh-ai-enhance-home-based-primary-care-and-operational-efficiency-at-bloom-healthcare/>

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